MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 16 OF 28

MFT 716

Communication Skills in Therapy

15 teaching weeks · 3 practicals · 3 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 706

> “Every therapeutic effect passes through communication.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 716 — Communication Skills in Therapy

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	**Communication Skills in Therapy**	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Every therapeutic effect passes through communication. This course refines the therapist's own instrument — listening, responding, questioning, silence, feedback, and the management of difficult conversations — and teaches the protocols you will hand to families.

How to use this book

Skills are built by repetition under observation. Each week has a **micro-skill** to practise, a **protocol** to teach to families, and a **transcript** to analyse. Record yourself weekly; the gap between what you think you said and what you said is the curriculum.

1. Course description

The course covers therapeutic communication at three levels: the **micro** (attending, reflecting, summarising, questioning), the **meso** (structuring difficult conversations, giving feedback, managing conflict), and the **macro** (written communication, interprofessional communication, and communication across language and culture).

It also teaches the **structured communication protocols** used with couples and families — speaker–listener, the Imago dialogue, negotiation formats, family meetings — and requires you to deliver them with fidelity.

2. Learning outcomes

#	Outcome	Assessed
LO1	Demonstrate micro-skills with fluency and intention	Observed session
LO2	Analyse communication at content, relationship and process levels	Transcript analysis
LO3	Teach two structured communication protocols with fidelity	Skills test
LO4	Conduct a difficult conversation (bad news, disclosure, confrontation)	Simulation
LO5	Adapt communication for children, interpreters and low-literacy contexts	Viva
LO6	Write clinical communication that is accurate and usable	Report portfolio
LO7	Identify and modify your own characteristic communication patterns	Reflective portfolio

3. Assessment

Component	Weight	Due
Transcript analysis (1,500 words)	20%	Week 6
Observed micro-skills session (30 min)	25%	Week 8
Protocol teaching test (speaker–listener or Imago)	25%	Week 11

Component	Weight	Due
Difficult conversation simulation (20 min)	30%	Week 14

4. The fifteen-week plan

Week	Topic	Practical
1	Communication as the medium of therapy	Self-recording baseline
2	Attending and non-verbal communication	Non-verbal drill
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Empathy: affective, cognitive, and its limits	Empathy accuracy exercise
5	Questioning and prompting	Question taxonomy drill
6	**Transcript analysis due**; content, relationship, process	Three-level analysis
7	Feedback: giving it, receiving it, teaching families to give it	Feedback lab
8	**Observed micro-skills session**	Assessed
9	Conflict communication and repair	Repair rehearsal
10	Speaker–listener and Imago dialogue	Protocol fidelity test prep
11	**Protocol teaching test**	Assessed
12	Difficult conversations: bad news, disclosure, confrontation	Simulation
13	Communication with children, interpreters and across literacy	Adaptation lab
14	**Difficult conversation assessment**	Assessed
15	Written and interprofessional communication	Report portfolio

Week 1 — Communication as the medium

Three levels of every message.

1. **Content** — what is said.
2. **Relationship** — what it implies about the relationship ("you are the kind of person who…").
3. **Process** — what is happening between the people while it is said.



Figure 1.1 — Five axioms of communication

The therapist's communication is an intervention. Every reflection selects something from the client's account and returns it amplified. There is no neutral way to summarise.

Baseline recording. Record ten minutes of your own session (consented) or a role-play. Count: questions, reflections, advice statements, interruptions, and seconds of silence you initiated. Most beginners are surprised by the advice count.

Week 2 — Attending and non-verbal communication

The SOLER posture (Egan): **S**it squarely, **O**pen posture, **L**ean slightly forward, **E**ye contact (culturally calibrated), **R**elax.

What families read. Pace, volume, facial expression, seating, who you look at, how you respond to distress, and whether you check your phone. A therapist who says "take your time" while glancing at the clock has said something else.

Cultural calibration. Eye contact signals respect in some cultures and challenge in others; physical distance varies; silence may indicate deference rather than resistance. Ask, or observe what the family does with each other.

Working with more than two people. Your attention is a resource that structures the family. Distribute it deliberately; narrate your choices ("I am going to come back to you, I have not forgotten"). Do not let the most articulate person become the narrator of the family.

> **DRILL**
> Triads. The therapist must convey three different messages using only non-verbal behaviour (welcome, "I need to interrupt", "take your time"). The client guesses each.

Week 3 — Listening

Paraphrase (content), **reflect** (feeling), **summarise** (pattern across people and time).

Reflection levels.

Level	Example	Effect
Simple	"You are angry about the money."	Shows attention; may stall
Complex	"You are angry about the money, and underneath that, frightened that you are the only one thinking about the future."	Adds meaning; deepens
Double-sided	"You want him to be more involved, and you also do not trust him to do it right."	Holds ambivalence

The 2:1 rule. Two reflections for every question, until it becomes automatic.

What blocks listening. Formulating your next question; evaluating; wanting to reassure; your own emotional reaction; diagnostic thinking that closes the account prematurely.

Week 4 — Empathy

Three components.

1. **Affective empathy** — feeling something in response.
2. **Cognitive empathy** — understanding the other's perspective.
3. **Empathic communication** — conveying that understanding so the other feels understood.

The therapist needs all three, and needs to keep the first from driving. Affective empathy without boundaries produces over-involvement; cognitive empathy without warmth produces technique; communication without either produces scripts.

Empathy is not agreement. "I understand why you would want to leave" is empathy. "You are right to want to leave" is agreement. The first opens exploration; the second closes it and creates a coalition.

Empathic accuracy is testable. Watch a recorded session and pause before each therapist response. Write what you think the client felt. Then check with the client (or with the recording's subsequent cues).

> **DRILL**
> In pairs, one person describes a minor grievance. The listener must respond with an empathic statement that contains no advice, no question and no story of their own. Three rounds each.

Week 5 — Questioning and prompting

Six families of question (from MFT 706), refined for communication work:

- **Open** — "What was that like?"
- **Circular** — "Who noticed first?"
- **Clarifying** — "When you say 'disrespectful', what did he actually do?"
- **Reflexive** — "If your daughter were listening, what would she say you both do?"
- **Scaling** — "How close are you, 0–10?"
- **Confronting (gently)** — "You say it does not matter, and you have mentioned it four times."

The prompt instead of the question. "Tell me more about that" often produces more than any question. Use prompts when the client is in flow; questions when you need to change direction.

Week 6 — Content, relationship, process

Transcript analysis exercise. Take fifty lines of a session transcript and mark:

- every **content** statement (topic, fact)
- every **relationship-level** message ("you always…", "you are the kind of…")
- every **process** moment (something audible happening between speakers: interruption, softening, joke, silence)

Then ask: what did the therapist respond to? Beginners respond to content and miss process. Advanced practitioners respond to process and use content as evidence.

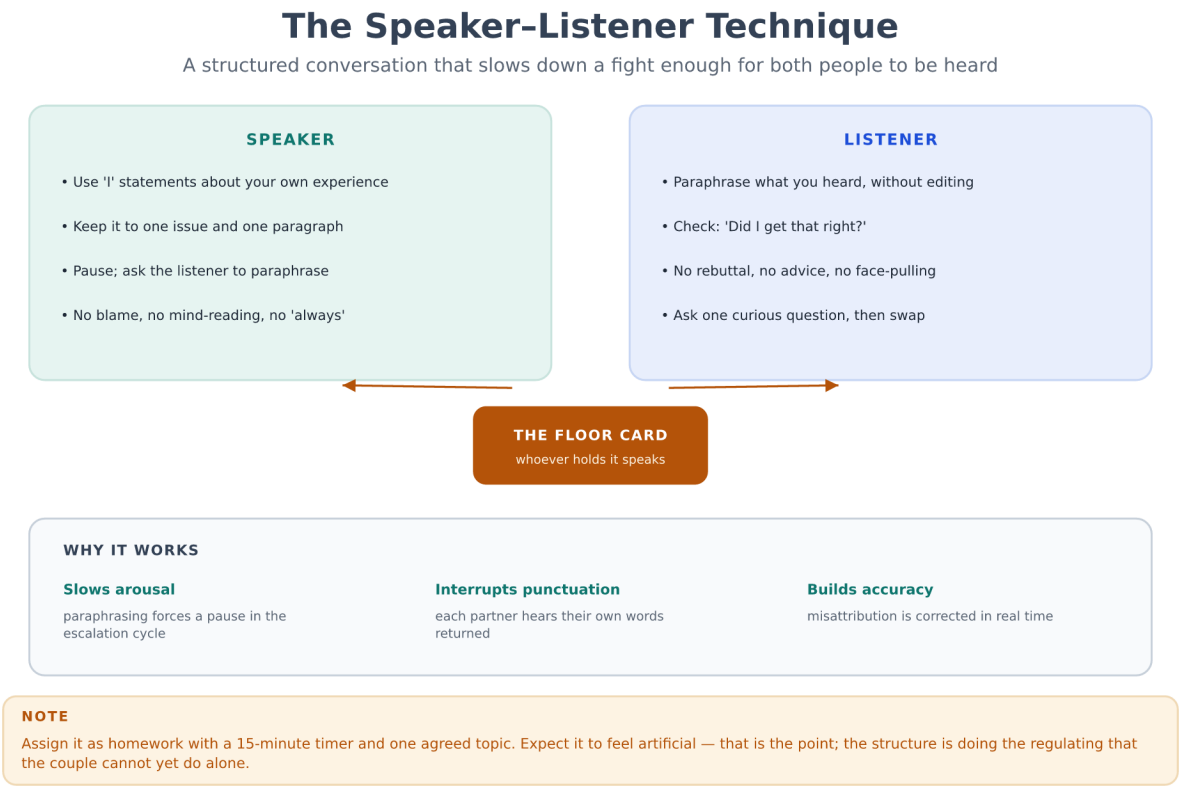


Figure 6.1 — The speaker–listener protocol

Week 7 — Feedback

Feedback to clients should be behavioural, specific, timely and owned: "I noticed your voice dropped when you said that" (not "you are passive-aggressive").

Feedback between family members is often the missing skill. Teach the four-part structure:

1. "When you ___" (behaviour, no adjectives)
2. "I feel ___" (feeling)
3. "Because ___" (meaning)
4. "What I would like is ___" (specific request)

Feedback to colleagues and supervisees: use the same discipline. "What I saw / what it did to me / what to keep / what to try" — never the sandwich, which trains people to hear praise as a warning.

Week 8 — Observed micro-skills session

Thirty minutes. Assessed on: attending; reflection ratio; complex reflections; question variety; one deliberate silence; one summary; and a clean close.

Week 9 — Conflict communication and repair

The escalation sequence: complaint → criticism → contempt → defensiveness → stonewalling.

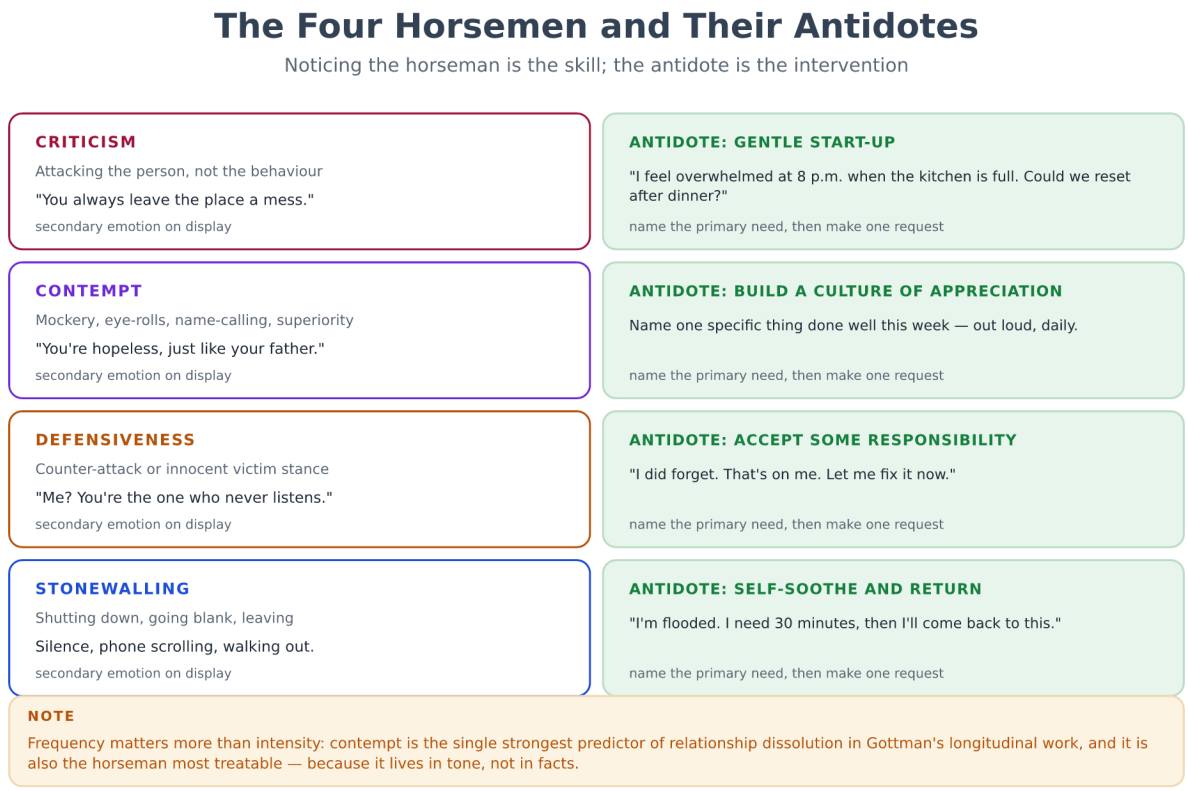


Figure 9.1 — The four horsemen and their antidotes

Repair is the skill that matters. Teach couples to use a repair phrase: "Can we start again?", "I am flooded, give me twenty minutes", "That came out wrong."

The therapist's repair. You will make mistakes: mis-summarise, miss a disclosure, take a side. Repair in the moment: "I got that wrong. Let me try again." Modelling repair is more powerful than describing it.

Week 10 — Protocols

Speaker–listener. A floor card, a timer, paraphrase before responding, then swap. Teach it, rehearse it in session, and set it as homework with one agreed topic.

Imago dialogue. Three steps: mirror ("so what I heard you say is…"), validate ("that makes sense because…"), empathise ("I imagine you feel…"). More demanding than speaker–listener, and more powerful with practised couples.

Negotiation format. Define one issue; each states a position; generate three options; choose one to trial for two weeks; review.

Fidelity matters. A protocol delivered loosely is a protocol that fails, and the couple will conclude that "communication exercises do not work". Practise until you can teach it in ten minutes without notes.

> **ASSESSED**
> Teach speaker–listener to a role-played couple in twelve minutes, then coach them through one round. Assessed on clarity, demonstration, coaching, and management of a partner who will not follow the rules.

Week 11 — Protocol teaching test

As above, assessed formally, with feedback.

Week 12 — Difficult conversations

Four types and their structures.

1. **Bad news** (a diagnosis, a death, a safeguarding outcome). Structure: warning shot ("I have some difficult news") → information in small chunks → check understanding → silence → emotional response → plan → follow-up. Do not fill the silence after the news.
2. **Disclosure** (a secret that must be shared, an affair, a risk). Structure: prepare the person → agree the content → be present → manage the response → plan.
3. **Confrontation** (a boundary violation, a safety concern, a colleague's conduct). Structure: describe the behaviour → its impact → the expectation → the consequence → the support available.
4. **Ending therapy when it is not working** (referral, rupture, incompetence). Structure: name the difficulty → own your part → offer a route → document.

The common failure is rushing to reassurance. "At least…" and "it will be fine" terminate the conversation the client needs to have.

Week 13 — Adaptation

Children. Short sentences, concrete language, check understanding by asking them to explain it back, and use a medium.

Interpreters. Speak to the family, first-person interpretation, brief and debrief (see MFT 708).

Low literacy. Avoid written-only instructions; use drawings, demonstration, and teach-back ("show me how you would do it").

Hearing or cognitive impairment. Reduce background noise, face the person, one idea per sentence, and confirm comprehension.

Second-language sessions. Slow down, avoid idiom, check meaning ("when you say 'respect', what does that include?").

Week 14 — Difficult conversation assessment

Twenty-minute simulation drawn from a bank (bad news, disclosure, confrontation, ending). Assessed on: structure, pacing, emotional attunement, clarity of the message, and a documented follow-up.

Week 15 — Written and interprofessional communication

Clinical notes. Behavioural, dated, factual, and signed. Avoid adjectives about people; avoid speculation.

Referral letters. One page: reason, relevant history, what you have tried, what you are asking for, and who to contact.

Reports for courts and schools. Answer the question asked; state the limits of your expertise; separate fact from opinion.

Interprofessional communication. Use SBAR (Situation, Background, Assessment, Recommendation). It is short, it survives interruption, and it is what other professions are trained to receive.

Glossary

Complex reflection — a reflection that adds meaning or feeling beyond the client's words. **Double-sided reflection** — a reflection that holds both sides of an ambivalence. **Empathic accuracy** — the degree to which a therapist correctly infers a client's experience. **Mirroring** — repeating back the other's words as the first step of a structured dialogue. **Process** — the relational layer of communication, distinct from content. **Repair** — an attempt to de-escalate and reconnect after a rupture. **SBAR** — Situation, Background, Assessment, Recommendation: a handover format. **SOLER** — a posture mnemonic for attending behaviour.

Self-assessment

Multiple choice

1. The recommended ratio of reflections to questions for beginners is: (a) 1:1 (b) 2:1 (c) 1:2 (d) 5:1

2. "You are right to want to leave him" is: (a) empathy (b) agreement (c) a reflection (d) a summary

3. A complex reflection: (a) repeats content (b) adds meaning or feeling (c) asks a question (d) gives advice

4. The main error in delivering bad news is: (a) pausing (b) rushing to reassurance (c) checking understanding (d) using plain language

5. SBAR stands for: (a) Situation, Background, Assessment, Recommendation (b) Summary, Behaviour, Action, Review (c) Safety, Boundaries, Alliance, Repair (d) Session, Block, Ask, Respond

6. In Imago dialogue the three steps are: (a) mirror, validate, empathise (b) ask, tell, repeat (c) listen, advise, conclude (d) reflect, question, summarise

7. Fidelity in teaching a communication protocol matters because: (a) it is required by codes (b) a loosely delivered protocol fails and discredits the method (c) it is faster (d) clients prefer it

8. Eye contact should be: (a) constant (b) culturally calibrated (c) avoided (d) used only with adults

9. The 2:1 rule exists to: (a) reduce interrogation and enrich material (b) fill time (c) avoid emotion (d) demonstrate technique

10. A referral letter should include: (a) the full case file (b) reason, relevant history, what you tried, the request, contact (c) only a diagnosis (d) your opinion of the family

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 a · 7 b · 8 b · 9 a · 10 b

Short answer

1. Describe the three levels of a message with one example of each.
2. Give three examples of a complex reflection on the same client statement.
3. What are the four steps of the feedback structure for family members?
4. Outline the structure for delivering bad news.
5. Name four adaptations for a family with limited literacy.

Essay (choose one, 1,800 words)

1. Analyse a transcript of your own session: what did you respond to, and what did you miss?
2. "Repair, not avoidance, is the mark of a skilled communicator." Discuss with clinical examples.
3. How should communication protocols be adapted for a family in which the couple does not share a first language?

Further reading

- Egan, G. (2013). The Skilled Helper (10th ed.). Cengage.
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- Gottman, J. M., & Silver, N. (2015). The Seven Principles for Making Marriage Work. Harmony.
- Hendrix, H., & Hunt, H. (2004). Getting the Love You Want. Holt.
- Silverman, J., Kurtz, S., & Draper, J. (2013). Skills for Communicating with Patients (3rd ed.). Radcliffe.

Next: MFT 717 — Psychopathology in Families